

Criteria	Yes	No
Diagnosis Discrepancy	☒	☐
Primary Tumor Site Discrepancy	☐	☒
HIPAA Discrepancy	☐	☒
Prior Malignancy History	☐	☒
Dual/Synchronous Primary Malignancy	☐	☒
Case is (circle): <u>QUALIFIED</u> / <u>DISQUALIFIED</u>		
Reviewer Initials: <u>RB</u> Date Reviewed: <u>4/17/12</u>		
<u>kw 4/30/12</u>		

Date of Procedure:
Date of Receipt:
Date of Report:
Account #:
Billing Type:
Additional Copy to:

Clinical Diagnosis & History:

Right thyroid nodule FNA: suspicious papillary thyroid cancer.

UUID: EAA7A141-7F59-4016-AC23-25F324A33DEA
TCGA-DJ-A3UT-01A-PR

Redacted



Specimens Submitted:

1: SP: Thyroid; total thyroidectomy

DIAGNOSIS:

1. SP: Thyroid; total thyroidectomy:

Tumor Type:

Three foci of papillary microcarcinoma

99% Follicular variant per TSS pathologist.

See attached TCGA Pathologic Diagnosis discrepancy Form. *for*

Histologic Grade:

Well differentiated

Mitotic Activity:

Not identified

Tumor Necrosis:

Not identified

Tumor Location:

Left lobe
Right lobe

Tumor Size:

Ranging from <0.1 cm to 0.9 cm

Blood Vessel Invasion:

Not identified

Extrathyroid Extension:

Not identified

Surgical Margins:

Free of tumor

Tumor Multicentricity:

Present (see above)

Adenoma(s) (away from the carcinoma):

Not identified

Non-Neoplastic Thyroid:

Exhibits nodular hyperplasia

Parathyroid Glands:

Not identified

ICD-O-3

carcinoma, papillary, follicular variant
8340/3

Site: thyroid, NOS

C73.9

5-2-12
RO

I ATTEST THAT THE ABOVE DIAGNOSIS IS BASED UPON MY PERSONAL EXAMINATION OF THE SLIDES (AND/OR OTHER MATERIAL), AND THAT I HAVE REVIEWED AND APPROVED THIS REPORT.

*** Report Electronically Signed Out ***

Gross Description:

1). The specimen is received fresh, labeled "total thyroidectomy" and consists of a thyroid weighing 14 g. The right lobe measures 5.0 x 2.9 by 1.2 cm, the left lobe measures 4.2 x 3.1 x 1.3 cm and the isthmus measures 1.5 x 0.8 by 0.3 cm. The external surface is covered by a thin fibrous capsule. The posterior surface of the specimen is inked purple, and the anterior is inked green. Sectioning reveals a well-circumscribed, tan nodule in the right lobe that measures 0.9 x 0.8 x 0.7 cm. The remaining thyroid parenchyma is red tan and beefy. Representative sections of the specimen have been submitted for . The remaining tissue is serially sectioned and entirely submitted.

Summary of sections:

N -- nodule
RL - right lobe
LL - left lobe
IS - isthmus

Summary of Sections:

Part 1: SP: Throid; total thyroidectomy

Block	Sect. Site	PCs
1	IS	2
4	LL	4
2	N	3
4	RL	6

TCGA Pathologic Diagnosis Discrepancy Form

V4.00

Instructions: The TCGA Pathologic Diagnosis Discrepancy Form should be completed when the pathologic diagnosis documented on the initial pathology report for a case submitted for TCGA is inconsistent with the diagnosis provided on the Case Quality Control Form completed for the submitted case.

Tissue Source Site (TSS): _____ TSS Identifier: _____ TSS Unique Patient Identifier: _____

Completed By (Interviewer Name on OpenClinica): _____ Completed Date: _____

Diagnosis Information

#	Data Element	Entry Alternatives	Working Instructions
1	Pathologic Diagnosis Provided on Initial Pathology Report	<i>Papillary microcarcinoma</i>	Provide the diagnosis/ histologic subtype(s) documented on the initial pathology report for this case. If the histology for this case is mixed, provide all listed subtypes.
2	Histologic features of the sample provided for TCGA, as reflected on the CQCF.	<i>Follicular variant of papillary carcinoma</i>	Provide the histologic features selected on the TCGA Case Quality Control Form completed for this case.

Discrepancy between Pathology Report and Case Quality Control Form

3	Provide the reason for the discrepancy between the pathology report and the TCGA Case Quality Control Form.	<i>Subcentimeter papillary carcinoma are labeled microcarcinomas. They can be follicular variant, clonical or others. Both diagnosis do not contradict each other.</i>	Provide a reason describing why the diagnosis on the initial pathology report for this case is not consistent with the diagnosis selected on the TCGA Case Quality Control Form.
4	Name of TSS Reviewing Pathologist or Biorepository Director	<i>/</i>	Provide the name of the pathologist who reviewed this case for TCGA.

I acknowledge that the above information provided by my institution is true and correct and has been quality controlled.

/ TSS Reviewing Pathologist or Biorepository Director

Date */*

I acknowledge that the above information provided by my institution is true and correct and has been quality controlled. The Attending Pathologist or the Department Chairman has been informed or is aware of the above discrepancy in diagnoses.

Principal Investigator Signature

Date

* Dr.

is the pathologist and the PI